Stanford HEALTH CARE

REFERRAL FOR CONSULT OR PROCEDURE

Digestive Health and Liver Clinic

Thank you for choosing Stanford Health Care.

STANFORD REFERRAL CENTER

PHONE: 877-254-3762

Physician Helpline: 866-742-4811

FAX: 650-320-9443

DATE: _____

of PAGES: _____

- ☐ Routine
- ☐ Urgent within a week
- ☐ Expedited (within a month)

REFERRING PROVIDER INFORMATION:

Referred by (MD): _____ This form completed by: _____

Medical Group: _____ EMAIL: _____

Phone: _____ Fax: _____ PCP: _____

Address: Cigna PPO _____ City: _____ ZIP: _____

_____

****

PATIENT INFORMATION (Please provide a copy of patient demographics/face sheet):

Last Name: Quinn _____ First Name: Avery _____ MI: _____

DOB: 03/12/1984 Phone: 530-555-0142 _____ Email: _____ Gender: ☐M ☐F

Patient's Address: _____

City/State/Zip: _____

Needs Interpreter? ☐Yes ☐No Language: _____

****

REFERRAL INFORMATION: (Please provide as much detail as possible to assist us in scheduling appropriately)

DIAGNOSIS (ICD CODE): K62.5 _____

REASON FOR REFERRAL: rectal bleeding _____

CONSULT REQUESTED

☐ Consultation only (Referring MD to prescribe therapy)

☐ Consult, treat and return to requesting MD when medically stable

- Would you like your patient to be seen by other services at SHC? ☐Yes / ☐No
  - ☐Surgery ☐Nutrition ☐Psychology
  - ☐Other service: _____

☐ Patient previously seen by another gastroenterologist for the same/similar problem:

MD name (if known): _____

PROCEDURE REQUESTED

☐ Screening Colonoscopy

Date of last colonoscopy: _____

☐ Surveillance Colonoscopy (history of polyps)

Date of last colonoscopy: _____

Histology of last polyp(s): _____

☐ Colonoscopy after: ☐ FIT Test + ☐ other: _____

☐ Upper GI endoscopy ☐ Capsule endoscopy

Endoscopic ultrasound: ☐ Upper ☐ Lower

☐ ERCP (requires consult)

☐ Balloon enteroscopy (requires consult)

☐ Endoscopic mucosal resection (requires consult)

MOTILITY STUDIES:

☐ Esophageal manometry

☐ 24hr esophageal impedance-pH

☐ Bravo pH ☐ Endoflip

Breath test: ☐ Lactulose ☐ Other: _____

☐ Anorectal Manometry

☐ Wireless capsule motility (SmartPill)

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Last Name: _____ First Name: _____ MI: _____ DOB: _____

FOR CONSULTS:

SUBSPECIALTY REQUESTED:
(for a detailed explanation please go to the Digestive Health Center pages on the Stanford Health Care website)

- ☐ General Gastroenterology
- ☐ Inflammatory Bowel Disease
- ☐ Liver
- ☐ Esophagus
- ☐ Motility and Functional Bowel Disease
- ☐ Neurogastroenterology
- ☐ Pelvic Health
- ☐ GI/Pain collaboration
- ☐ Celiac Sprue, Intestinal disorders and Allergic disorders
- ☐ Nutrition/Intestinal Failure/Intestinal Transplant
- ☐ Pancreas/Biliary
- ☐ Therapeutic/Advanced Endoscopy
  Consider procedure:_____
- ☐ High risk cancer screening/genetics

FOR PROCEDURES:

REQUIRED FOR PROCEDURE REQUESTS:
Does patient have any of the following? (Check if yes)
☐ LVAD ☐ Pacemaker* ☐ Defibrillator*
☐ Taking aspirin/Plavix/warfarin/anticoagulants*:
- ☐ Can stop medication for procedure
- ☐ Cannot stop medication for procedure
- ☐ Defer to cardiology/neurology/hematology

*MD managing pacemaker/defibrillator/meds:
Name:_____
Tel:_____ Fax:_____

SEDATION/PROCEDURAL CONSIDERATIONS:
- ☐ Patient had prior problems with sedation or anesthesia.
- ☐ Patient is on chronic pain mediations
- ☐ Patient has a history of alcohol or drug abuse
- ☐ Patient has significant cardio-pulmonary disease
- ☐ Patient has significant liver or kidney disease
- ☐ Patient has insulin-dependent diabetes
- ☐ Patient has other significant systemic disease

****

DOCUMENTATION REQUIRED *(Please fax with this form):*

- Recent/relevant typed test results, ie: history & physical, previous colonoscopies/MRI/CT/X-Rays, labs
- Last Colonoscopy and pathology report if appropriate if requesting colonoscopy
- Recent/relevant typed clinic notes
- Proof of Insurance
- Authorization information (*if required*)

Physician Requested (optional): _____

If Requested Physician is not available, is it OK to schedule with another qualified physician? ☐ Yes / ☐ No

ClinLab
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

Specimen Number	Patient ID	Control Number	Account Number	Account Phone Number	Route

Patient Last Name	Account Address

Patient First Name	Patient Middle Name
Jeff	

Patient SS#	Patient Phone	Total Volume

Age (Y/M/D)	Date of Birth	Sex	Fasting
46 v.o.		Male	Yes

Patient Address	Additional Information

Date and Time Collected	Date Entered	Date and Time Reported	Physician Name	NPI	Physician ID

Tests Ordered
CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel

General Comments

TESTS	RESULT	FLAG	UNITS	REFERENCE INTERVAL	LAB
CBC, Platelet Ct, and Diff					
Hematocrit	40.6		%	40.8 - 51.9	07
Hemoglobin	14.2		g/dL	14.6 - 17.8	07
Red Blood Cell Count	8.4		x10E6/uL	4.69 - 6.07	07
White Blood Cell Count	7.4		x10E3/uL	3.2 - 10.6	07
RDW	13.0		%	10.8 - 14.1	07
MCV	82.6		fL	77.8 - 94.0	07
MCH	27.4		pg	26.5 - 32.6	07
MCHC	34.4		g/dL	32.7 - 36.9	07
Imm.Granulocytes (Absolute)	6.2		x10E3uL	1.3 - 7.0	07
Granulocytes (Percent)	57.2		%	44.0 - 76.0	07
Eosinophils (Absolute)	0.3		x10E3uL	0.0 - 0.4	07
Eosinophils (Percent)	5.2		%	0.0 - 6.0	07
Basophil (Absolute)	0.08		x10E3uL	0.0 - 0.1	07
Basophil (Percent)	1.7		%	0 - 2	07
Monocytes (Absolute)	0.64		x10E3uL	0.2 - 0.7	07
Monocytes (Percent)	6.9		%	4 - 8.9	07
Lymphocytes (Absolute)	2.26		x10E3uL	0.8 - 3.1	07
Lymphocytes (Percent)	37.2		%	15 - 43	07
Platelets (Absolute)	388		x10E3uL	177 - 406	07
Liver Function					
Aspartate aminotransferase (AST)	223	High	U/L	8 - 43	07
Alanine Aminotransferase (ALT)	257	High	U/L	7 - 45	07
Iron and Total Iron Binding					
Serum Iron	2300	Very High	ug/dL	40 - 155	07
Total Iron Binding Capacity	328		ug/dL	250 - 400	07
Transferrin Saturation	72	Very High	%	20 - 50	07

PRELIMINARY INTERPRETATION:

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.